

# DYNA ERYTHROMYCIN TABLET

MAL19962105AZ

## DESCRIPTION:

### TABLET

Colour : Pink  
Shape : Round / Biconvex  
Coating : Film-coated

## EACH TABLET CONTAINS:

Erythromycin Stearate \* 420 mg equivalent to Erythromycin ..... 250 mg

\* Approximate potency equivalent to not less than 600 µg of Erythromycin per mg.

## PHARMACODYNAMICS:

Erythromycin is a macrolide antibiotic. It interferes with bacterial protein synthesis by binding to the 50S subunit of ribosomes. It is bacteriostatic or bactericidal depending on its concentration and the type of organism. Activity increases with increases in pH up to about pH 8.5. The minimum inhibitory concentrations of Erythromycin for sensitive micro-organisms have been reported to range from less than 0.1 to about 2 µg per ml.

## PHARMACOKINETICS:

Erythromycin base is unstable in gastric acid and food may reduce the absorption of the base or the stearate. After absorption, Erythromycin diffuses readily into most body fluids. In the absence of meningeal inflammation, low concentrations are normally achieved in the spinal fluid but passage of the drug across the blood-brain-barrier increases in meningitis. In the presence of normal hepatic function, Erythromycin is concentrated in the liver and excreted in the bile; the effect of hepatic dysfunction on excretion of Erythromycin by the liver into the bile is not known. After oral administration, < 5% of the activity of the administered dose can be recovered in the urine. Erythromycin crosses the placental barrier but foetal plasma levels are low.

## INDICATION:

Upper and lower respiratory tract infection, ear and eye infections, gastrointestinal, skin and soft tissue infections. Gonorrhoea, Syphilis and Whooping cough.

## RECOMMENDED DOSAGE:

Adults: Usual dose: 250 mg 4 times daily. Dosage may be increased up to 4 g/day according to the severity of the infection.

Children: 30-50 mg/kg/day, in divided doses. For more severe infections, this dose may be doubled.

Optimum serum levels of Erythromycin may be reached if taken in the fasting state or immediately before meals.

## ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

## CONTRAINDICATION:

Patient hypersensitive to Erythromycins.

## WARNING AND PRECAUTIONS:

Erythromycin is principally excreted by the liver. Caution should be exercised in administering the antibiotic to patients with impaired hepatic function. There have been reports of hepatic dysfunction, with or without jaundice, occurring in patients receiving oral Erythromycin products. Risk-benefit should be considered in patients with cardiac arrhythmias (history of), QT prolongation or loss of hearing.

Rare cases of serious cardiovascular adverse events including deaths, cardiac arrests, torsades de pointes and other ventricular arrhythmia have been observed when used in patients taking concomitant terfenadine.

There have been reports of infantile hypertrophic pyloric stenosis (IHPS) occurring in infants following erythromycin therapy. In one cohort of 157 newborns who were given erythromycin for pertussis prophylaxis, seven neonates (5%) developed symptoms of non-bilious vomiting or irritability with feeding and were subsequently diagnosed as having IHPS requiring surgical pyloromyotomy. Since erythromycin may be used in the treatment of conditions in infants which are associated with significant mortality or morbidity (such as pertussis or chlamydia), the benefit of erythromycin therapy needs to be weighed against the potential risk of developing IHPS. Parents and caregivers should be informed to contact their physician if vomiting and/ or irritability with feeding occurs.

In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP)], DYNA ERYTHROMYCIN TABLET should be discontinued immediately and appropriate treatment should be urgently initiated.

## DRUG INTERACTIONS:

Erythromycin may interact with astemizole and terfenadine, resulting in potentially serious adverse cardiovascular effects; and therefore these antihistamines are contraindicated in patients receiving Erythromycin. Other medications e.g. digoxin; ergotamine; ototoxic medications; penicillins; lovastatin; alfentanil; coumarin-derivative or indandione-derivative anticoagulants; hydantoin anticonvulsants; drugs metabolised in the P450 cytochrome system like hexobarbital; oral antidiabetic agents; trizolam; carbamazepine; chloramphenicol; cyclosporine; other hemolytics; other hepatotoxic medications; lincomycins; methenamine; methotrexate; or xanthines, especially theophylline may affect the use of Erythromycin.

## PREGNANCY AND LACTATION:

The safety of Erythromycin for use during pregnancy has not been established. Erythromycin crosses the placental barrier. Erythromycin also appears in breast milk. Caution should be exercised when Erythromycin is administered to a nursing woman.

**SIDE EFFECTS/ADVERSE REACTIONS:**

The most frequent adverse effects of Erythromycin preparations are gastrointestinal, e.g. abdominal cramping and discomfort, and are dose-related. Nausea, vomiting and diarrhoea occur infrequently with usual oral doses. During prolonged or repeated therapy, there is a possibility of overgrowth of non-susceptible bacteria or fungi. If such infections occur, the drug should be discontinued and appropriate therapy instituted. Mild allergic reactions, e.g. urticaria and other skin rashes have occurred. Serious allergic reactions, including anaphylaxis, have been reported. There have been isolated reports of reversible hearing loss occurring chiefly in patients with renal insufficiency and in patients receiving high doses of Erythromycin. Pseudomembranous has been rarely reported in association with Erythromycin therapy. There have been isolated reports of transient central nervous system side effects including confusion, hallucinations, seizures and vertigo; however, a cause and effect relationship has not been established. Occasional case reports of cardiac arrhythmias such as ventricular tachycardia have been documented in patients receiving Erythromycin therapy. There have been isolated reports of other cardiovascular symptoms such as chest pain, dizziness and palpitations; however, a cause and effect relationship has not been established.

**Postmarketing Experience:**

**Gastrointestinal Disorder:** infantile hypertrophic pyloric stenosis.

**Skin and Subcutaneous Tissue Disorders**

Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP).

**SYMPTOMS AND TREATMENT OF OVERDOSE:**

In case of overdose, Erythromycin should be discontinued. Overdose should be handled with the prompt elimination of unabsorbed drug and all other appropriate measures. Erythromycin is not removed by peritoneal dialysis or hemodialysis.

Allergic reactions associated with acute overdose should be handled in the usual manner, that is, by the administration of adrenaline, corticosteroids and antihistamines as indicated and the prompt elimination of unabsorbed drug, in addition to all needed supportive measures.

**PACKAGING/PACK SIZE(S):**

Blister pack of 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK**

**KEEP OUT OF REACH OF CHILDREN**

**MANUFACTURER/PRODUCT REGISTRATION HOLDER:**

**DYNAPHARM (M) SDN. BHD. 198001011897(65683-V)**

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